

Name: Daniel S May | DOB: 10/11/1989 | MRN: 10105654130 | PCP: Joshua W Clark, NP | Legal Name:
Daniel S May

Telemedicine - Jul 10, 2026

with Stella Lee, MD at Brigham and Women's Otolaryngology Clinic



Notes from Care Team



Progress Notes by Stella Lee, MD at 7/10/2026 4:00 PM



BRIGHAM AND
WOMEN'S HOSPITAL



HARVARD
MEDICAL SCHOOL

Otolaryngology-Head and Neck Surgery

45 Francis Street, Boston MA 02115

Name: Daniel May

MRN: 48968218

Chief Complaint: No chief complaint on file.

Referred by: Clark, Joshua W, NP

PCP: Clark, Joshua W, NP

7/10/2026

It was a pleasure to see Daniel S May in follow-up for severe symptoms including 20 lb Unintentional weight loss, bright red blood in the stool, thick mucus problems, left ear discomfort, facial pressure, severe hypertension requiring emergency room visit.

History of Present Illness

A 36-year-old male with persistent left ear discomfort, chronic sinonasal symptoms, and gastrointestinal complaints seen for evaluation of ongoing ENT issues.

Left ear discomfort has been present for 1-2 years, described as a sensation of something lodged in the ear, with associated crackling sounds when moving the jaw. The left ear is reported as the most bothersome, with intermittent subtle pain and persistent discomfort. Self-examination with an ear camera revealed a yellowish-white mass and drainage. Multiple visits to an otolaryngologist have not identified a clear etiology. An ear tube was placed but did not alleviate symptoms.

Sinonasal symptoms have persisted for over 1 year, including daily need to blow the nose due to clear, sticky, stringy mucus. Mucus production is most pronounced in the morning and recurs throughout the day. No recent thick or green mucus has been noted. Upper sinus pressure near the eyes and dry skin around the eyes have been reported. Allergy testing previously showed dust mite sensitivity, but no recent allergic symptoms have been experienced. Dr. Brennan indicated that his immune function was normal.

Sleep disruption is attributed to ENT symptoms, particularly the need to blow the nose and ear discomfort. Difficulty falling asleep and waking early for work have contributed to fatigue and inability to work for the past week.

Gastrointestinal complaints include frequent stomach noises after eating, occasional blood in stool, and unintentional weight loss exceeding 20 pounds. Dietary changes have included increased yogurt, honey, probiotics, and prebiotic fibers, but frequent consumption of processed foods persists. Persistent white coating is removed daily with a tongue scraper. Bottled water is consumed.

Elevated blood pressure has been noted at multiple appointments, with a peak home reading of 191/115 prompting an emergency room visit. Blood pressure at the ER was 145/90. Associated symptoms included nausea and sweating while driving, with near-emesis.

SOCIAL HISTORY

He works as a real estate manager for early education child care.

PAST MEDICAL, SURGICAL, FAMILY & SOCIAL HISTORY:

Below data was updated and reviewed.

Past Medical History: has a past medical history of Trauma to eye.

Past Surgical History: has a past surgical history that includes Sinus surgery (Bilateral).

Family History: family history is not on file.

Social History: He works as a real estate manager for early education child care. No exposure to dust, fumes, chemicals.

Results

The following results were reviewed by myself.

Imaging

- CT scan of the sinuses: 07/09/2026, Sinuses appear normal with no significant abnormalities. Mild inflammation noted in the right maxillary sinus. No evidence of sinus infection or mass.
- CT scan of the left ear and mastoid: 07/09/2026, Minimal fluid present in the left mastoid air cells. No other significant abnormalities identified.

Diagnostic Testing

- EKG: EKG was performed in the emergency department; specific results were not disclosed or reviewed during this encounter.

Assessment & Plan

Daniel S May is a very pleasant 36-year-old gentleman with an unintentional 20-lb weight loss, blood in stool, nausea, a history of significant worsening symptoms where he is unable to function normally, come asleep, or work. We discussed the complexity of his symptoms, a large part of his workup that is important, however, is GI, I will touch base with his physician.

1. Severe red-flag gastrointestinal symptoms unintentional weight loss >20 pounds and blood in stool reported. Abnormal GI symptoms including nausea and stool changes noted.

- I will send an urgent note to Dr. Owyang, his GI physician, I see he is scheduled in August for endoscopy but he may need intervention sooner.

2. Left ear discomfort

Persistent left ear discomfort with fluid noted in mastoid on imaging. Symptoms may be related to underlying gastrointestinal dysfunction. Imaging from 2021 reviewed and compared.

- Advised to avoid further invasive interventions until diagnosis clarified.

3. Sinus symptoms

Sinuses appear healthy on imaging with only minor right maxillary inflammation. Symptoms not consistent with sinus source of symptoms. No significant sinus disease seen on imaging from yesterday.

4. Hypertension

Consistently elevated blood pressure readings reported at multiple visits and ER. No acute intervention discussed.

- Continue regular blood pressure monitoring. Follow up with primary care for management. Patient has a nurse practitioner who follows him, discuss that he may be best served by obtaining a physician for his overall care.

Stella Lee, MD

Sinus and Endoscopic Skull Base Surgery

Associate Surgeon

Otolaryngology-Head and Neck Surgery

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I personally spent a total of 36 minutes on care for this patient on the date of the encounter. This includes face-to-face time during the visit as well as non face-to-face time spent on chart review, radiographic imaging, documentation, testing, and care coordination and excluding the time spent in performance of the procedure.

I obtained verbal consent from the patient or their proxy to record this visit for purposes of producing a draft of the encounter documentation.